

	Problems/Challenges	Observations	Detrimental impact	Solutions
1	Responsibility assigned to less appropriate entities	Designated to Manning agency (India) In-house medical team (HQ Country) Third-party administrators (Non-Indian country)	Increased crew dissatisfaction and crew attrition. Longer resolution time. Increased medical bills. Increased indirect expenses.	India based specialized third-party agency. Domain expertise in maritime health, insurance and logistics.
2	Lack of Geographic Reach & Cultural Understanding	Unrealistic expectations are set to the crew/provider. Unable to understand the practical difficulties. Lack of understanding of the medical infrastructure and the treatment providers ecosystem.	Prolonged communication gap. Treatment comes to a halt or delayed for extended period. Increased treatment cost. Worsening of patient's medical condition & instances of complications.	Expansive network for providers. Regional field officers & partners. Pre-negotiated treatment rates.
3	Absence of a "Policy Framework" for Medical Provider Selection	Designated to a PEME facility & not a "Treating Facility". Crew is given an open hand to choose a facility of his/her choice. Distant treatment provider is suggested.	Increased instances of false/ingenuine claims. Manipulated & overstating claims. Fabricated diagnosis. Phantom billing. Unnecessary or excessive hospitalization & treatment.	Centralized provider selection policy including accreditation standards, emergency response time, treatment quality, and pricing transparency. Tiered hospital empanelment list. Periodic reviews and audits of providers.
4	Differences in Medical Treatment Protocols	The Non-Indian medical doctors/Employer's HQ medical team professional may suggest a particular exam or a treatment plan which is not viable or possible in India E.g : Ozone Injections, Holter Monitor Test, Cryotherapy, etc	Treatment comes to a sudden halt. Unscheduled delay in patient recovery. Increased direct/indirect losses to the employer.	Second opinions. Standard treatment guideline library for the top 20 most common seafarer injuries/conditions.

5	Insufficient information on varied & ever changing treatment cost structures	Very often an In-patient procedure is either "under-approved" or "over-approved". Few treatments/drugs are reimbursed despite they might not fall under the obligation of the employer/insurance. Indian alternative medicine might be more suitable and cost-effective. Upcoding.	Rise of disputes. Monetary loss for both the crew as well as the employer. Scope for faster recovery option is ignored.	Treatment cost intelligence dashboard. (By region & Condition) Pre-treatment cost estimates & approvals. Extensive data driven tools to flag cost deviation.
6	Lack of Seamless Case Reporting & Submissions	Crew becomes the single source of information regarding the process and he/she might not be very responsive due to the illness. There were multiple instances of manipulated submissions of bills/reports. Language barrier or handwritten records makes the process more effort burning.	Delayed decision making. Hinder care coordination. Loss of time, money & efforts. Inaccurate record maintenance.	Central digital case management platform. Mobile APP.
7	Absence of Robust Verification & Field Authentication	Instances of fake claims are on high. The treating facility is operating without valid licenses and delivering sub-standard care. Treating facility & the crew together exploit the claim benefits for their monetary gains.	Fraudulent claims. Inflated bills. Fake provider entities. Unbundling.	Local field agents. Geo-tagged medical visit logs. Time-stamped records.
8	Siloed Health Records Management	Crew's health records from the PEME center is issued without following the due procedures. PEME records are not assessed by the Ship clinic seriously. Most of the times the treating facility in India is not shared with the PEME records & Ship/Shore clinic findings.	Chances of exploiting the insurance benefit is increased. Fabricated diagnosis.	Integrating with existing database. Single platform digital health record management.
9	Other Operational Barriers	Language barriers. Time zone differences. Logistics challenges. Communication with remote/rural providers.	Longer time taken to achieve complete recovery. Higher cost to the employer.	